INTERNATIONAL REVIEW OF PSYCHIATRY
2024, VOL. 36, NO. 8, 869–878
https://doi.org/10.1080/09540261.2024.2357669

Taylor & Francis
Taylor & Francis Group

RESEARCH ARTICLE

OPEN ACCESS Check for updates

Personal psychedelic experience of psychedelic therapists during training: should it be required, optional, or prohibited?

Daniel Villiger

Uehiro Centre for Practical Ethics, University of Oxford, Oxford, UK

ABSTRACT

Personal psychedelic experience is common among psychedelic therapists and often considered to be a necessary aspect of training: only personal psychedelic experience allows psychedelic therapists to properly guide patients through their own psychedelic experience, to truly understand that experience, and to help them integrate it into their lives. But is this really true? The present paper examines the value of therapists' personal psychedelic experience, why this value may be higher than that of personal experience with other psychotropic drugs, and whether it justifies a requirement of personal psychedelic experience for psychedelic therapists. The analysis, which also considers the literature on therapists' personal experience with the mental disorder being treated or with psychotherapy, concludes that the current evidence does not justify making personal psychedelic(-like) experience a requirement for psychedelic therapists. However, because therapists' personal psychedelic experience can be valuable to both therapists and patients, and because the likelihood of harm is very low, psychedelic therapists should be given the opportunity to have a psychedelic experience during their training.

ARTICLE HISTORY
Received 4 March 2024
Accepted 14 May 2024

KEYWORDS
Psychedelic-assisted therapy; personal psychedelic experience; psychedelic ethics; training; personal therapy

Introduction

Psychedelics have made an impressive comeback in mental health care. In the last decade, an increasing number of studies have investigated the therapeutic effects of classic serotonergic psychedelics, including lysergic acid diethylamide (LSD), psilocybin, and N,N-dimethyltryptamine (DMT).[1] The present results are promising and suggest efficacy in the treatment of major depressive disorder (Carhart-Harris, Bolstridge, et al., 2016; Carhart-Harris et al., 2018, 2021; Goodwin et al., 2022; Palhano-Fontes et al., 2019; Raison et al., 2023; Reiff et al., 2020; Rotz et al., 2023; Sanches et al., 2016), anxiety with and without a life-threatening illness (Gasser et al., 2014; Griffiths et al., 2016; Holze et al., 2023; Johnson & Griffiths, 2017; Reiff et al., 2020; Ross et al., 2016), and alcohol and smoking cessation (Bogenschutz et al., 2015; Garcia-Romeu et al., 2019; Johnson, Garcia-Romeu, & Griffiths, 2017; Johnson, Garcia-Romeu, Johnson, et al., 2017; Krebs & Johansen, 2012). Meta-analyses and systematic reviews underscore the potential benefits of psychedelics in mental health care and suggest that psychedelics can be safely used in the clinical context (Andersen et al., 2021; Bender & Hellerstein, 2022; Haikazian et al., 2023; Ko et al., 2023; Maia et al., 2022; Romeo et al., 2020; Roscoe & Lozy, 2022; Simonsson et al., 2023; Wheeler & Dyer, 2020).

Among the classic serotonergic psychedelics, psilocybin is currently receiving the most attention. In February 2024, there were 84 ongoing clinical studies using psilocybin, including four phase 3 trials (ClinicalTrials, 2024). For DMT and LSD, there were seven and four ongoing clinical trials, respectively. In addition to their use in clinical trials, psychedelics have begun to be used clinically in non-research contexts too. Countries such as Canada, Switzerland, and Israel grant psychiatrists special permission to use psychedelics under certain conditions, and in 2023 Australia has become the first country to generally allow psilocybin for treatment (Ducharme, 2023; Haridy, 2023). Taken together, we can already observe significant clinical use of psychedelics, and it is very likely that this use will continue to increase in the coming years.

CONTACT Daniel Villiger daniel.villiger@philosophy.ox.ac.uk Uehiro Centre for Practical Ethics, Faculty of Philosophy, University of Oxford, Suite 1 Littlegate House, St Ebbe's Street, Oxford OX1 1PT, UK
© 2024 The Author(s). Published by Informa UK Limited, trading as Taylor & Francis Group
This is an Open Access article distributed under the terms of the Creative Commons Attribution License (http://creativecommons.org/licenses/by/4.0/), which permits unrestricted use, distribution, and reproduction in any medium, provided the original work is properly cited. The terms on which this article has been published allow the posting of the Accepted Manuscript in a repository by the author(s) or with their consent.

not, because personal psychedelic use is mainly accused of compromising scientific objectivity, and while scientific objectivity is essential in psychedelic research, it is less so in psychedelic practice (cf. Kious et al., 2023). Besides, if the same were true for psychedelic therapist, it would be difficult to explain why many patients are likely to prefer a psychedelic therapist with personal psychedelic experience to one without.

Taken together, therapists' personal psychedelic experiences can be valuable to both therapists and patients, and the likelihood of harm is very low. Because of that, it seems reasonable to give psychedelic therapists a legal opportunity to have a psychedelic experience during their training (with the same inclusion criteria as psychedelic studies with healthy participants).

Conclusion

This paper analyzed whether personal psychedelic experience of psychedelic therapists during training should be required, optional, or prohibited. The analysis found no evidence demonstrating that a therapist's personal psychedelic(-like) experience improves therapeutic effects or reduces negative effects. Furthermore, it found no conclusive evidence that therapists' personal experience with the mental disorder being treated or with psychotherapy improves treatment outcomes, suggesting that the same may be true for therapists' personal psychedelic(-like) experience. Therefore, a requirement of personal psychedelic(-like) experience for psychedelic therapists is not justified. However, as personal psychedelic experiences can be valuable and have a low risk profile, they should be offered to therapists during PAT training.

On a final note, while the line of argument presented in this paper refers to classic serotonergic psychedelics, it can also be applied to other kinds of psychoactive therapeutics in which the acute subjective effects may play some role (e.g. ketamine or MDMA): Only if there is evidence that personal experience with such a substance improves treatment outcomes can a requirement for such experience as a therapist be justified. At the same time, if the substance has a very low likelihood of harm and self-reports indicate that therapists' personal experience with it is professionally valuable, trainees should be given the opportunity to try it.

Notes

1. In addition, studies with 3,4-methylenedioxymethamphetamine (MDMA), which is sometimes also classified as a psychedelic substance, have shown treatment benefits for post-traumatic stress disorder (Mithoefer et al., 2018; Ot'alora G et al., 2018), including phase 3 trials (Mitchell et al., 2021, 2023), and for social anxiety among patients on the autism spectrum (Danforth et al., 2018). However, the focus of this paper will be on the classic serotonergic psychedelics.
2. Of course, pharmacotherapy and psychotherapy are also combined in other treatments (e.g. psychotherapy with antidepressants). However, the combination is usually not mandatory, and the two therapies are not that much intertwined.
3. Note that the method of therapeutic touch, which dates back to PAT practices in the 1960s and has long been regarded best practice in modern PAT studies, has come under scrutiny in recent years. Devenot et al. (2022) argue that research on the subject is insufficient to justify it on scientific grounds, considering that therapeutic touch can (unintentionally) cross boundaries.
4. For research examining this claim, it would also be interesting to analyze whether/how the three types of personal experiences (i.e. the mental disorder being treated, psychotherapy, and psychedelics) interact with each other in PAT.

Disclosure statement

No potential conflict of interest was reported by the author(s).

Funding

This work was supported by the Swiss National Science Foundation under grant number 222239.

ORCID

Daniel Villiger http://orcid.org/0000-0003-0851-624X

References

Aday, J. S., Skiles, Z., Eaton, N., Fredenburg, L., Pleet, M., Mantia, J., Bradley, E. R., Fernandes-Osterhold, G., & Woolley, J. D. (2023). Personal psychedelic use is common among a sample of psychedelic therapists: Implications for research and practice. *Psychedelic Medicine*, *1*(1), 27–37. https://doi.org/10.1089/psymed.2022.0004

Andersen, K. A., Carhart-Harris, R., Nutt, D. J., & Erritzoe, D. (2021). Therapeutic effects of classic serotonergic psychedelics: A systematic review of modern-era clinical studies. *Acta Psychiatrica Scandinavica*, *143*(2), 101–118. https://doi.org/10.1111/acps.13249

Barrett, M. S., & Berman, J. S. (2001). Is psychotherapy more effective when therapists disclose information about themselves? *Journal of Consulting and Clinical Psychology*, *69*(4), 597–603. https://doi.org/10.1037/0022-006X.69.4.597

Beauchamp, T. L., & Childress, J. F. (2013). *Principles of biomedical ethics* (7th ed.). Oxford University Press.

Bender, D., & Hellerstein, D. J. (2022). Assessing the risk–benefit profile of classical psychedelics: A clinical review of second-wave psychedelic research. *Psychopharmacology*, *239*(6), 1907–1932. https://doi.org/10.1007/s00213-021-06049-6

Bogenschutz, M. P., Forcehimes, A. A., Pommy, J. A., Wilcox, C. E., Barbosa, P. C. R., & Strassman, R. J. (2015). Psilocybin-assisted treatment for alcohol dependence: A proof-of-concept study. *Journal of Psychopharmacology*, *29*(3), 289–299. https://doi.org/10.1177/0269881114565144

Brennan, W., Jackson, M. A., MacLean, K., & Ponterotto, J. G. (2021). A qualitative exploration of relational ethical challenges and practices in psychedelic healing. *Journal of Humanistic Psychology*, 1–31. https://doi.org/10.1177/00221678211045265

Carhart-Harris, R. L., Bolstridge, M., Day, C. M. J., Rucker, J., Watts, R., Erritzoe, D. E., Kaelen, M., Giribaldi, B., Bloomfield, M., Pilling, S., Rickard, J. A., Forbes, B., Feilding, A., Taylor, D., Curran, H. V., & Nutt, D. J. (2018). Psilocybin with psychological support for treatment-resistant depression: Six-month follow-up. *Psychopharmacology*, *235*(2), 399–408. https://doi.org/10.1007/s00213-017-4771-x

Carhart-Harris, R. L., Bolstridge, M., Rucker, J., Day, C. M. J., Erritzoe, D., Kaelen, M., Bloomfield, M., Rickard, J. A., Forbes, B., Feilding, A., Taylor, D., Pilling, S., Curran, V. H., & Nutt, D. J. (2016). Psilocybin with psychological support for treatment-resistant depression: An open-label feasibility study. *The Lancet. Psychiatry*, *3*(7), 619–627. https://doi.org/10.1016/S2215-0366(16)30065-7

Carhart-Harris, R. L., & Friston, K. J. (2019). REBUS and the anarchic brain: Toward a unified model of the brain action of psychedelics. *Pharmacological Reviews*, *71*(3), 316–344. https://doi.org/10.1124/pr.118.017160

Carhart-Harris, R. L., Giribaldi, B., Watts, R., Baker-Jones, M., Murphy-Beiner, A., Murphy, R., Martell, J., Blemings, A., Erritzoe, D., & Nutt, D. J. (2021). Trial of psilocybin versus escitalopram for depression. *The New England Journal of Medicine*, *384*(15), 1402–1411. https://doi.org/10.1056/NEJMoa2032994

Carhart-Harris, R. L., Kaelen, M., Bolstridge, M., Williams, T. M., Williams, L. T., Underwood, R., Feilding, A., & Nutt, D. J. (2016). The paradoxical psychological effects of lysergic acid diethylamide (LSD). *Psychological Medicine*, *46*(7), 1379–1390. https://doi.org/10.1017/S0033291715002901

ClinicalTrials. (2024). *Ongoing clinical trials with psilocybin*. https://clinicaltrials.gov./search?intr=psilocybin&aggFilters=status:not%20rec%20act

Culbreth, J. R. (2000). Substance abuse counselors with and without a personal history of chemical dependency. *Alcoholism Treatment Quarterly*, *18*(2), 67–82. https://doi.org/10.1300/J020v18n02_05

Danforth, A. L., Grob, C. S., Struble, C., Feduccia, A. A., Walker, N., Jerome, L., Yazar-Klosinski, B., & Emerson, A. (2018). Reduction in social anxiety after MDMA-assisted psychotherapy with autistic adults: A randomized, double-blind, placebo-controlled pilot study. *Psychopharmacology*, *235*(11), 3137–3148. https://doi.org/10.1007/s00213-018-5010-9

de Vos, J. A., Netten, C., & Noordenbos, G. (2016). Recovered eating disorder therapists using their experiential knowledge in therapy: A qualitative examination of the therapists' and the patients' view. *Eating Disorders*, *24*(3), 207–223. https://doi.org/10.1080/10640266.2015.1090869

Devenot, N., Tumilty, E., Buisson, M., McNamee, S., Nickles, D., & Ross, L. K. (2022, March 9). *A precautionary approach to touch in psychedelic-assisted therapy—Bill of health*. https://blog.petrieflom.law.harvard.edu/2022/03/09/precautionary-approach-touch-in-psychedelic-assisted-therapy/

Dubinski, K. (2020, December 10). Some doctors, therapists get Health Canada permission to use magic mushrooms. CBC News. https://www.cbc.ca/news/canada/london/some-doctors-therapists-get-health-canada-permission-to-use-magic-mushrooms-1.5834485

Ducharme, J. (2023, February 8). Psychedelics may be part of U.S. medicine sooner than you think. *Time*. https://time.com/6253702/psychedelics-psilocybin-mdma-legalization/

Earleywine, M., Low, F., Altman, B. R., & De Leo, J. (2023). How important is a guide who has taken psilocybin in psilocybin-assisted therapy for depression? *Journal of Psychoactive Drugs*, *55*(1), 51–61. https://doi.org/10.1080/02791072.2022.2047842

Elliott, M., & Ragsdale, J. (2020). Mental health professionals with mental illnesses: A qualitative interview study. *The American Journal of Orthopsychiatry*, *90*(6), 677–686. https://doi.org/10.1037/ort0000499

Emmerich, N., & Humphries, B. (2023). Is the requirement for first-person experience of psychedelic drugs a justified component of a psychedelic therapist's training? *Cambridge Quarterly of Healthcare Ethics*, 1–10. https://doi.org/10.1017/S0963180123000099

Engel, L. B., Thal, S. B., & Bright, S. J. (2022). Psychedelic forum member preferences for carer experience and consumption behavior: Can "trip sitters" help inform psychedelic harm reduction services? *Contemporary Drug Problems*, *49*(4), 356–368. https://doi.org/10.1177/00914509221121420

Forstmann, M., & Sagioglou, C. (2021). How psychedelic researchers' self-admitted substance use and their association with psychedelic culture affect people's perceptions of their scientific integrity and the quality of their research. *Public Understanding of Science*, *30*(3), 302–318. https://doi.org/10.1177/0963662520981728

Garcia-Romeu, A., Davis, A. K., Erowid, F., Erowid, E., Griffiths, R. R., & Johnson, M. W. (2019). Cessation and reduction in alcohol consumption and misuse after psychedelic use. *Journal of Psychopharmacology*, *33*(9), 1088–1101. https://doi.org/10.1177/0269881119845793

Gasser, P., Holstein, D., Michel, Y., Doblin, R., Yazar-Klosinski, B., Passie, T., & Brenneisen, R. (2014). Safety and efficacy of lysergic acid diethylamide-assisted psychotherapy for